

Meridian Spine & Rehabilitation Centre

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Consultant in Spinal Medicine: Dr Priya Raghavan MBBS FRCS (Tr & Orth)

UPDATED CLINICAL RECORDS AND REVISED TREATMENT PLAN

Patient	Joseph Thompson	Date of birth	04 March 1987
Claim reference	CLAIM-5182	Date of accident	22 February 2026
Instructing party	Claims handling, personal injury	Report date	17 September 2026
Mechanism	Rear-end road traffic collision, stationary vehicle struck at approx. 30 mph	Records enclosed	Pages 1-4

Why these records are being sent now. Mr Thompson was reviewed in clinic on 15 September 2026 following a deterioration in his symptoms that began in late July. The clinical picture has changed materially since the last report of 06 May 2026. The working diagnosis of an uncomplicated whiplash-associated disorder no longer accounts for his presentation. Imaging now confirms a disc protrusion at L4/L5 with early nerve root involvement. The prognosis, the treatment plan and the return-to-work assumptions all require revision, and four items of treatment need fresh funding authorisation before they can be scheduled.

1. Summary of changes since the report of 06 May 2026

Item	Previously recorded (06 May 2026)	Current position (15 Sep 2026)
Diagnosis	Whiplash-associated disorder grade II; cervical soft tissue injury	WAD grade II (resolving) PLUS L4/L5 disc protrusion with right L5 radiculopathy
Prognosis to recovery	4 to 6 months from accident; substantial recovery expected by Aug 2026	12 to 18 months from this review; full resolution no longer assumed
Pain score (VAS, worst 24h)	4 / 10	7 / 10
Neurological signs	None recorded	Reduced right ankle reflex; altered sensation right L5 dermatome
Work status	Phased return, expected full duties by July 2026	Unfit for current duties; light duties only, no manual handling above 5 kg
Treatment	Physiotherapy, 8 sessions, completed	See revised plan, section 4

2. Clinical findings on examination, 15 September 2026

On inspection Mr Thompson stands with a mild antalgic list to the left. Lumbar flexion is restricted to approximately 40 degrees with pain reproduced at end range. Straight leg raise is positive on the right at 35 degrees, reproducing pain radiating below the knee into the lateral calf and dorsum of the foot. The right ankle jerk is diminished compared with the left. Power is 4+/5 in right extensor hallucis longus. There is subjective altered sensation across the right L5 dermatome. Cervical spine range of movement has improved substantially since May and is now close to normal, consistent with the expected trajectory of the original soft tissue injury. There are no red flag features: no saddle anaesthesia, no bladder or bowel disturbance, no unexplained weight loss, no night pain of sinister character.

Mr Thompson reports that the low back and right leg symptoms began to intensify in the last week of July 2026, approximately five months after the index accident, without any new injury, fall or precipitating event. He describes a gradual onset over roughly ten days, with symptoms now constant rather than intermittent. He has been sleeping poorly and taking co-codamol most nights.

3. Imaging

MRI lumbar spine without contrast, 08 September 2026, Meridian Imaging, reported by Dr Alan Whitcombe FRCR.

Level	Finding	Significance
L3/L4	Mild disc desiccation. No protrusion. Canal and foramina patent.	Age-consistent degenerative change. Not symptomatic.
L4/L5	Right posterolateral disc protrusion measuring 6 mm, indenting the thecal sac and contacting the traversing right L5 nerve root. Moderate right foraminal narrowing.	Accounts for the current radicular symptoms and the diminished right ankle reflex.
L5/S1	Disc height preserved. No protrusion. Facet joints unremarkable.	Normal.
Overall	No fracture, no listhesis, no marrow signal abnormality, conus normal.	No structural instability; no surgical emergency.

There is no prior lumbar imaging available for direct comparison. The first MRI obtained in this claim was of the cervical spine only (11 March 2026), which is consistent with the presenting complaint at that time.

4. Revised treatment plan – funding authorisation required

None of the four items below is covered by the treatment authorisation currently on file. That authorisation was issued on 04 April 2026 and covered an initial course of eight physiotherapy sessions, which has been completed and closed. Each item below requires a fresh decision before it can be booked. Indicative costs are quoted at our standard schedule rates and are valid for 60 days.

#	Item	Provider / setting	Sessions	Indicative cost	Clinical rationale
1	Targeted lumbar rehabilitation programme	Meridian Spine, outpatient	16 over 12 weeks	GBP 2,240.00	Graded loading and motor control work directed at the L4/L5 segment. Distinct from the completed cervical programme.
2	Right L5 transforaminal epidural steroid injection (image guided)	Meridian Spine, day case	1, with option of a second at 6 weeks	GBP 1,150.00 per procedure	Diagnostic and therapeutic. Expected to reduce radicular pain sufficiently to allow item 1 to proceed.
3	Pain management psychology, CBT-informed	Northgate Psychological Services	8 over 10 weeks	GBP 1,000.00	Screening scores indicate developing fear-avoidance and sleep disturbance, both established predictors of chronicity in this population.
4	Vocational assessment and workplace ergonomic review	Return-to-work services, on site at employer	1 assessment plus 1 review	GBP 780.00	Current role involves repeated manual handling. A return to full duties is not safe without a formal assessment of accommodations.

Total indicative cost if all four items are authorised: **GBP 5,170.00** (assuming a single injection under item 2). If a second injection is clinically indicated, add GBP 1,150.00.

5. Prognosis

In my opinion the L4/L5 protrusion is attributable to the index accident on the balance of probabilities. The mechanism is consistent, the interval to symptom onset is within the range commonly seen for a delayed presentation of a traumatic disc injury, and Mr Thompson had no material history of back complaints before February 2026. I note his general practitioner record contains a single entry for mechanical back pain in 2019 which resolved within three weeks and required no imaging or referral; I do not regard that episode as

significant.

With the treatment plan at section 4 delivered in full, I would expect substantial improvement over 12 to 18 months from the date of this review, that is by approximately March 2028 at the outer end. Without that treatment the outlook is materially worse and I would expect a significant risk of persistent radicular symptoms. I estimate the risk of requiring lumbar decompression surgery at approximately 15 to 20 per cent within three years. Should surgery be required, a further 6 to 9 months of recovery should be assumed on top of the above.

I would recommend a further review at six months, or sooner if there is any progression of neurological deficit.

6. Effect on work and earnings

The loss of earnings schedule currently on file is out of date and should not be relied upon. That schedule was prepared in May 2026 on the assumption of a full return to normal duties by July 2026. That assumption is no longer correct.

Period	Work capacity	Basis
22 Feb 2026 - 21 Apr 2026	Unfit for all work	Acute phase, cervical injury
22 Apr 2026 - 26 Jul 2026	Phased return, reduced hours, light duties	Recorded in employer occupational health notes
27 Jul 2026 - present	Unfit for current role. Sedentary and light duties only. No manual handling above 5 kg. No prolonged standing beyond 20 minutes.	Current clinical findings, section 2
Forward view	Restrictions expected to persist for at least 9 months. A return to unrestricted duties cannot be assumed until item 4 of the treatment plan is complete and reviewed.	Prognosis, section 5

Mr Thompson's employer has indicated informally that light duties can be accommodated for a limited further period only. A vocational assessment is therefore time sensitive.

7. Enclosures

Doc	Description	Date	Pages
A	MRI lumbar spine, full radiology report, Dr A Whitcombe FRCR	08 Sep 2026	2
B	Clinic letter, review consultation, Dr P Raghavan	15 Sep 2026	3
C	Physiotherapy discharge summary, cervical programme, 8 sessions	19 Jun 2026	1
D	Pain and function questionnaire scores (ODI, HADS, TSK)	15 Sep 2026	2
E	Indicative cost schedule for the four treatment items above	16 Sep 2026	1
F	GP record extract, 2018 to date, redacted to relevant entries	12 Sep 2026	6

8. Outcome measures recorded at review

Measure	06 May 2026	15 Sep 2026	Direction
Oswestry Disability Index	22% (moderate)	44% (severe)	Deteriorated
VAS pain, worst in 24 hours	4 / 10	7 / 10	Deteriorated
HADS anxiety	6 (normal)	12 (moderate)	Deteriorated
HADS depression	4 (normal)	9 (mild)	Deteriorated
Tampa Scale of Kinesiophobia	31 (low)	43 (elevated)	Deteriorated
Neck Disability Index	30% (moderate)	12% (minimal)	Improved

The divergence between the neck score, which has improved as expected, and every lumbar and psychological measure, which has worsened, is the clearest indication that a second and separate problem has emerged

rather than the original injury simply failing to settle.

9. Declaration

I am a Consultant in Spinal Medicine at the Meridian Spine & Rehabilitation Centre. I have been Mr Thompson's treating clinician since 11 March 2026. This letter is prepared in my capacity as treating clinician and is not an independent medico-legal report under Part 35. Where I have expressed an opinion on causation I have set out my reasons. I confirm that the contents of this letter are true to the best of my knowledge and belief.

Signed	Dr Priya Raghavan
Position	Consultant in Spinal Medicine, MBBS FRCS (Tr & Orth)
GMC number	7 412 886
Date	17 September 2026

Action requested from the claims handler

	Action	Why it cannot wait
1	Authorise, decline or part-authorise the four treatment items at section 4.	Nothing can be booked against the closed April authorisation. Each week of delay extends the prognosis.
2	Instruct a revised loss of earnings schedule.	The schedule on file assumes a July 2026 return to full duties, which did not happen.
3	Consider whether the reserve set on this claim remains adequate.	The prognosis has moved from 4-6 months to 12-18 months and now carries a 15-20% surgical risk.
4	Confirm whether an independent medical examination is to be instructed.	If one is intended, it should be arranged before the injection at item 2 alters the clinical picture.

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